

Started on	Sunday, 12 February 2023, 8:57 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:57 PM
Time taken	14 secs
Marks	0.00/35.00
Grade	0.00 out of 100.00

Question 1

Not answered

Marked out of 1.00

Which one among the following is an early sign of opioid withdrawal?

Select one:

- ☐ Cold turkey
- ☐ Stomach cramps
- ☐ Diarrhoea
- ☐ Nausea
- ☐ Anxiety

Your answer is incorrect.

Early signs and symptoms of opioid withdrawal starts within 6-12 hours for short-acting drugs, and 24- 36 hours for longer-acting drugs. Symptoms include muscle aches, anxiety and agitation, sleep issues, running nose and yawning. Nausea and vomiting, diarrhoea, goose bumps, stomach cramps, depression or dysphoria and intense drug cravings start later (late onset symptoms)

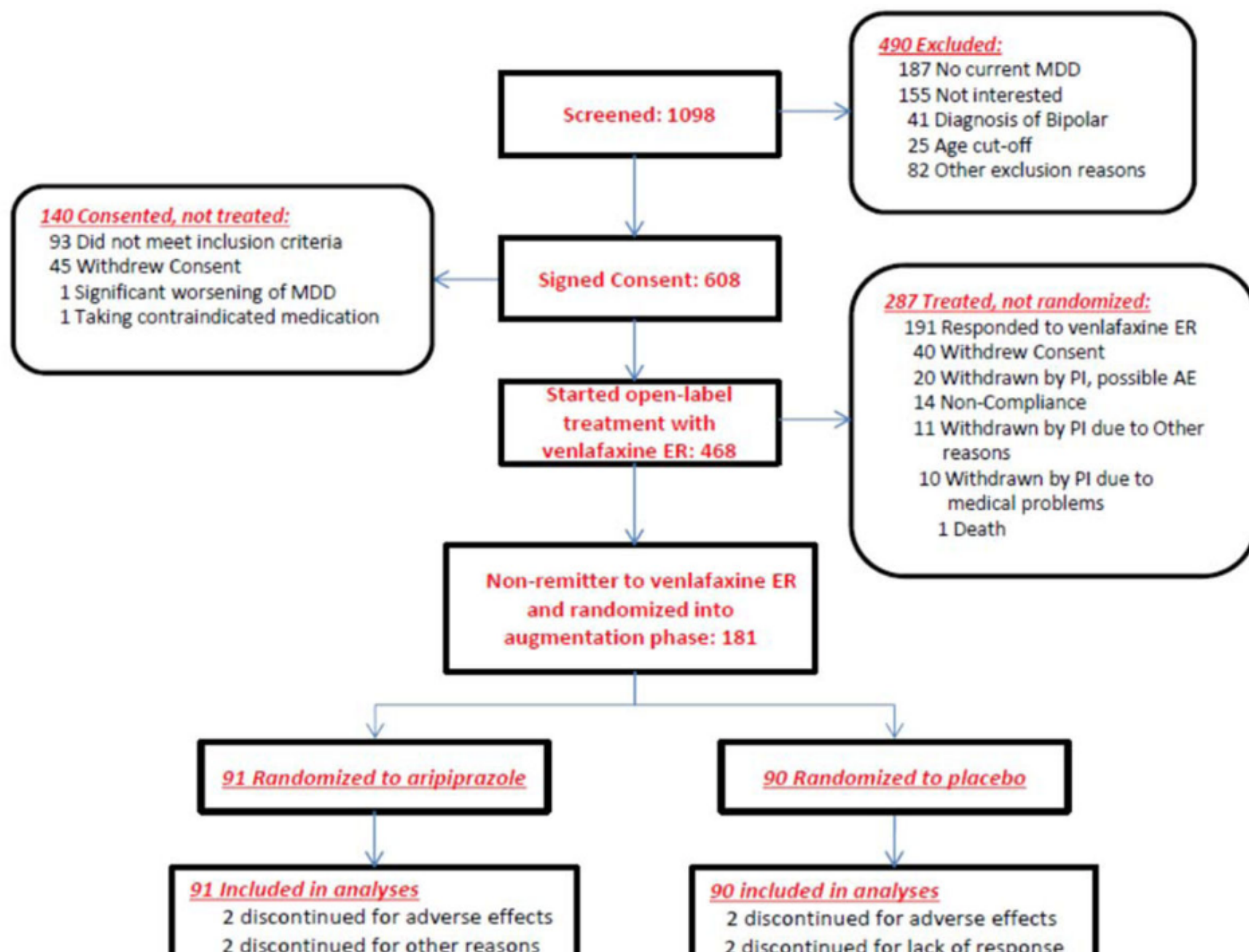
The correct answer is: Anxiety

Question 2

Not answered

Marked out of 1.00

In a clinical study, 468 participants older than 60 with current major depressive episode were initially treated with venlafaxine extended-release (ER); 96 (20.5%) did not complete this open phase, 191 (40.8%) remitted, and 181 (38.7%) did not remit and were randomized to 12 weeks of double-blind augmentation with aripiprazole or placebo. Remission, defined as Montgomery-Asberg Depression Rating Scale scores ≥ 10 , was assessed along with resolution of suicidal ideation, and safety and tolerability with cardiometabolic and neurological measures. Analyses were conducted according to the intention-to-treat principle. Older adults on aripiprazole had a higher remission rate than those on placebo (44% versus 29%; odds ratio [OR]=2.0, 95% CI 1.1–3.7, $p=0.03$; number needed to treat [NNT]=6.6 [95% CI 3.5–81.8]). Overall, remission was stable during 12 additional weeks of continuation treatment. (<http://europepmc.org/articles/pmc4690746>). What kind of study is this?



Select one:

- ☐ Uncontrolled trial
- ☐ Naturalistic observational study
- ☐ Cohort study
- ☐ Qualitative study
- ☐ RCT

40 completed as remitter
47 completed as nonremitter

3 discontinued for other reasons
26 completed as remitter
57 completed as nonremitter

40 Remitted and eligible for
continuation phase

26 Remitted and eligible for
continuation phase

Your answer is incorrect.

This is a randomised controlled trial. The attached flowchart is a CONSORT diagram.

The correct answer is: RCT

38 Included in analyses
5 Relapsed
33 Did not relapse

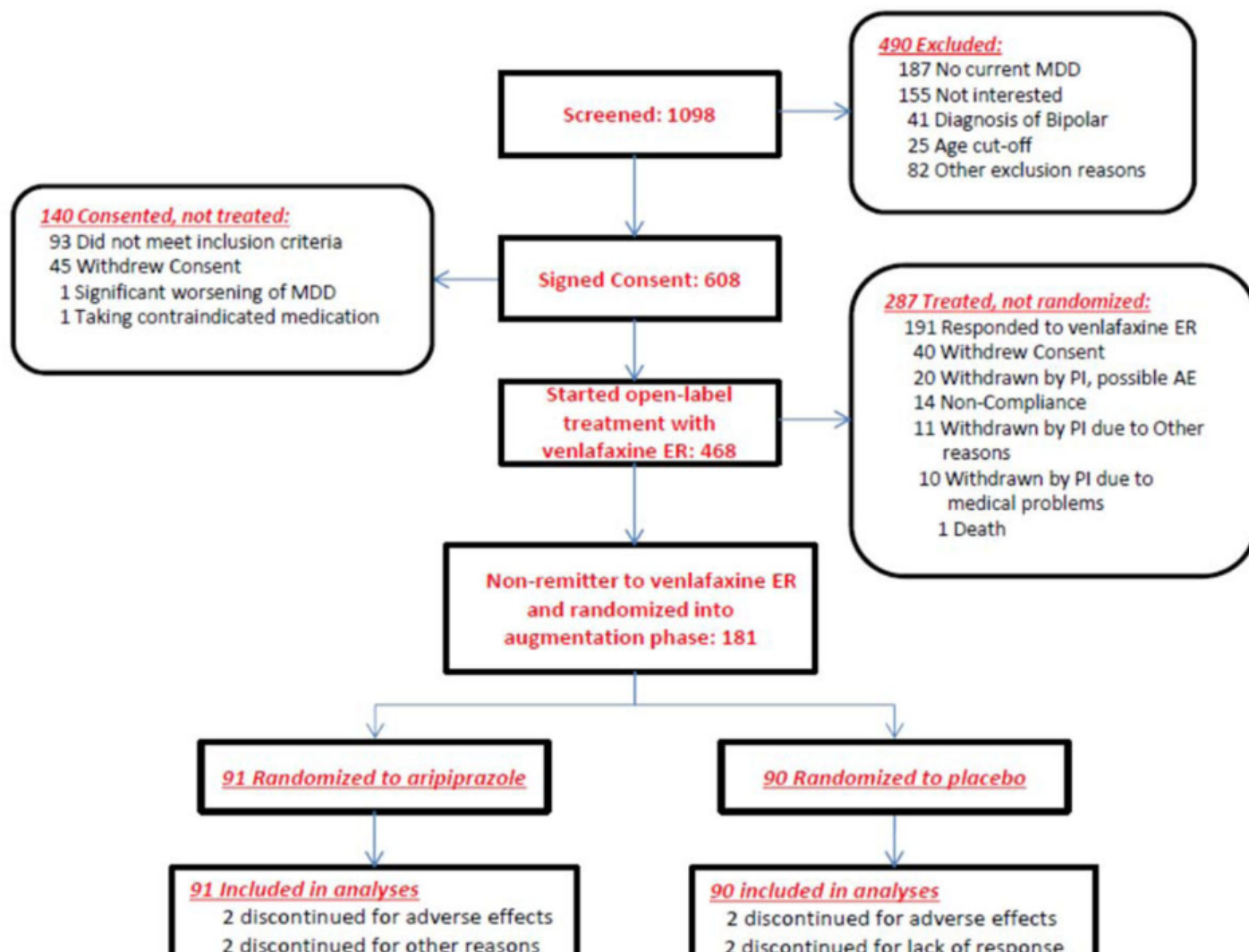
25 Included in analyses
2 Relapsed
23 Did not relapse

Question 3

Not answered

Marked out of 1.00

In a clinical study, 468 participants older than 60 with current major depressive episode were initially treated with venlafaxine extended-release (ER); 96 (20.5%) did not complete this open phase, 191 (40.8%) remitted, and 181 (38.7%) did not remit and were randomized to 12 weeks of double-blind augmentation with aripiprazole or placebo. Remission, defined as Montgomery-Asberg Depression Rating Scale scores ≥ 10 , was assessed along with resolution of suicidal ideation, and safety and tolerability with cardiometabolic and neurological measures. Analyses were conducted according to the intention-to-treat principle. Older adults on aripiprazole had a higher remission rate than those on placebo (44% versus 29%; odds ratio [OR]=2.0, 95% CI 1.1–3.7, $p=0.03$; number needed to treat [NNT]=6.6 [95% CI 3.5–81.8]). Overall, remission was stable during 12 additional weeks of continuation treatment. (<http://europepmc.org/articles/pmc4690746>). What is the primary comparison evaluated in this study?



Select one:

- ☐ Venlafaxine + placebo vs aripiprazole + placebo
- ☐ Venflaxine + aripiprazole vs placebo
- ☐ Arpipiprazole vs venlafaxine
- ☐ Venflaxine + aripiprazole vs placebo + venlafaxine
- ☐ Aripiprazole vs placebo

40 completed as remitter

47 completed as non-remitter

3 discontinued for other reasons

26 completed as remitter

57 completed as nonremitter

40 Remitted and eligible for continuation phase

26 Remitted and eligible for continuation phase

Your answer is incorrect.

This study tests the efficacy and safety of aripiprazole augmentation of venlafaxine therapy for older adults with depression.

The correct answer is: Venflaxine + aripiprazole vs placebo + venlafaxine

38 Included in analyses

5 Relapsed

33 Did not relapse

25 Included in analyses

2 Relapsed

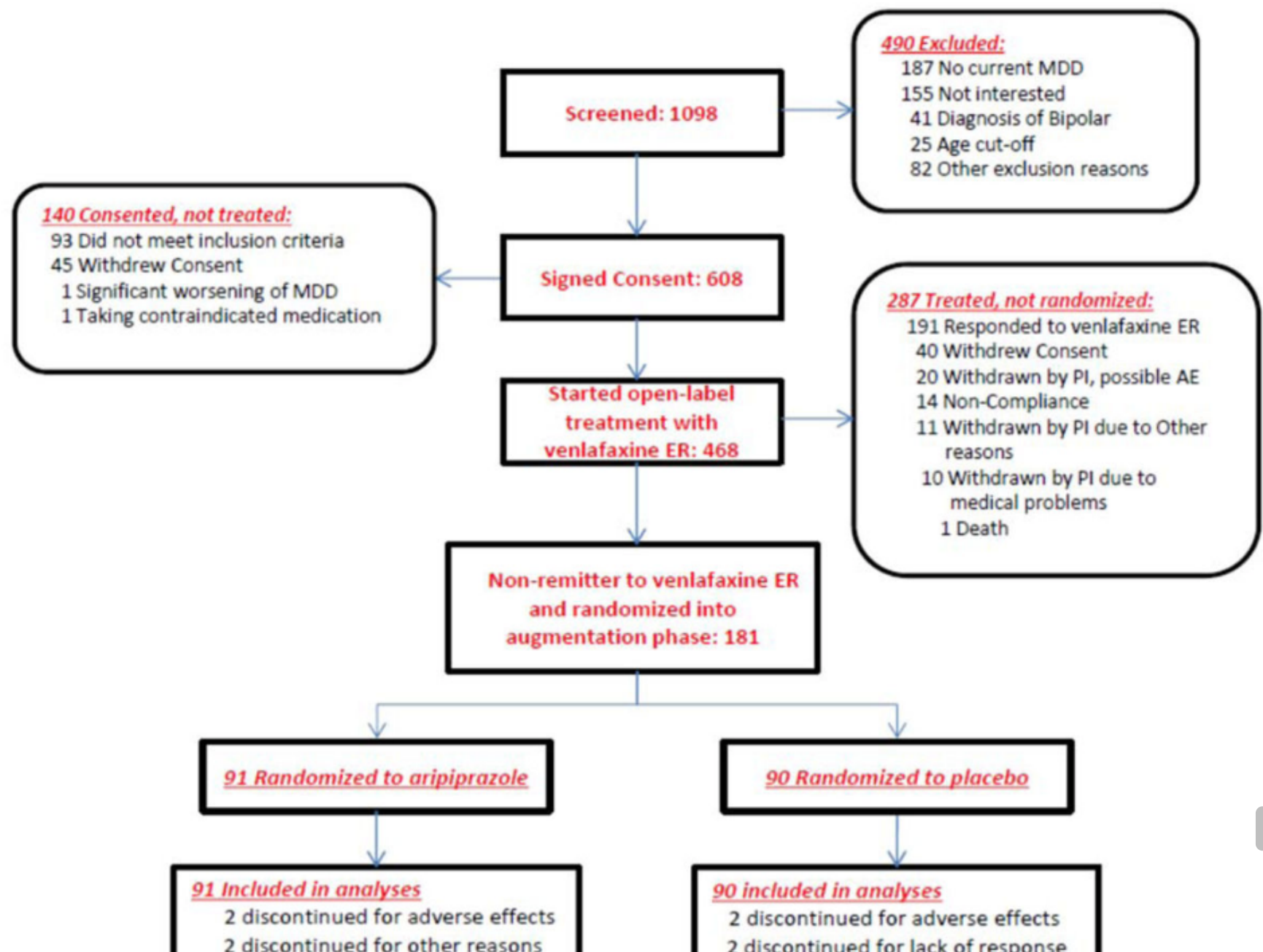
23 Did not relapse

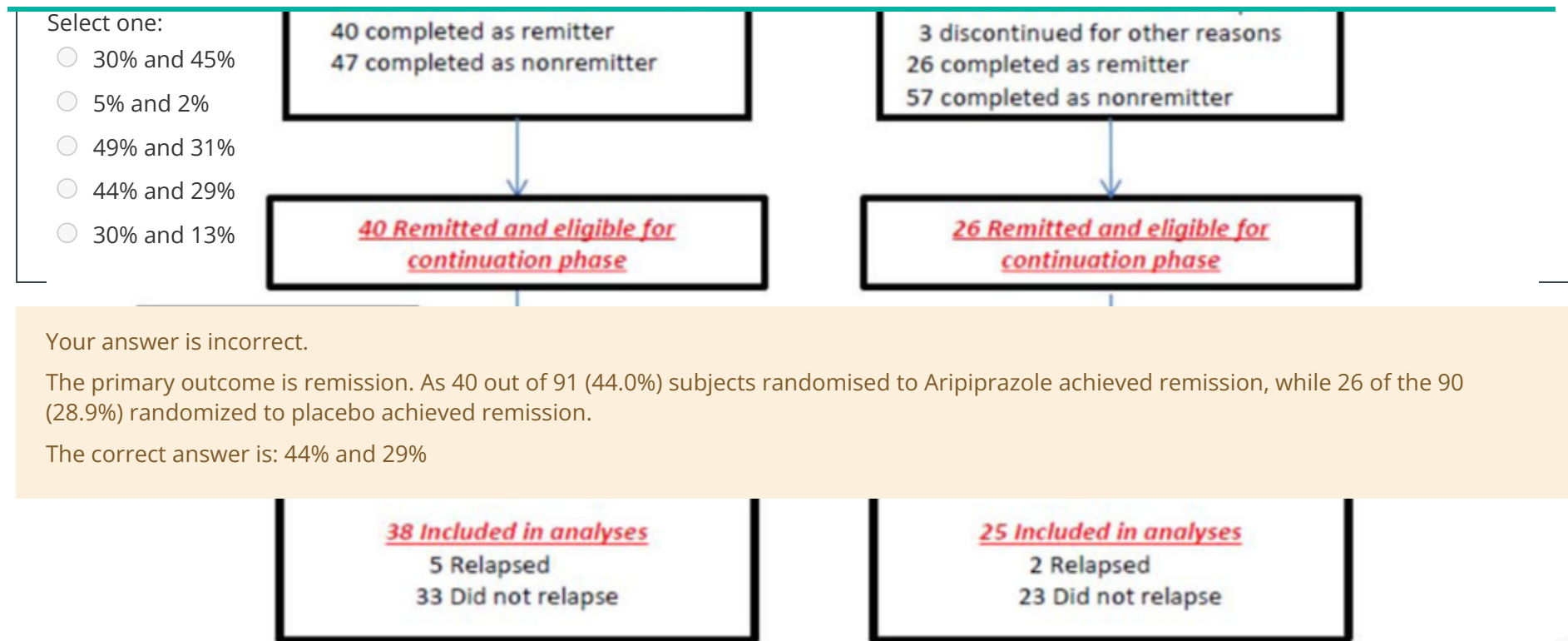
Question **4**

Not answered

Marked out of 1.00

In a clinical study, 468 participants older than 60 with current major depressive episode were initially treated with venlafaxine extended-release (ER); 96 (20.5%) did not complete this open phase, 191 (40.8%) remitted, and 181 (38.7%) did not remit and were randomized to 12 weeks of double-blind augmentation with aripiprazole or placebo. Remission, defined as Montgomery-Asberg Depression Rating Scale scores ≥ 10 , was assessed along with resolution of suicidal ideation, and safety and tolerability with cardiometabolic and neurological measures. Analyses were conducted according to the intention-to-treat principle. Older adults on aripiprazole had a higher remission rate than those on placebo (44% versus 29%; odds ratio [OR]=2.0, 95% CI 1.1–3.7, $p=0.03$; number needed to treat [NNT]=6.6 [95% CI 3.5–81.8]). Overall, remission was stable during 12 additional weeks of continuation treatment. (<http://europepmc.org/articles/pmc4690746>). The probability of achieving the primary outcome in those to aripiprazole and placebo is respectively





Your answer is incorrect.

The primary outcome is remission. As 40 out of 91 (44.0%) subjects randomised to Aripiprazole achieved remission, while 26 of the 90 (28.9%) randomized to placebo achieved remission.

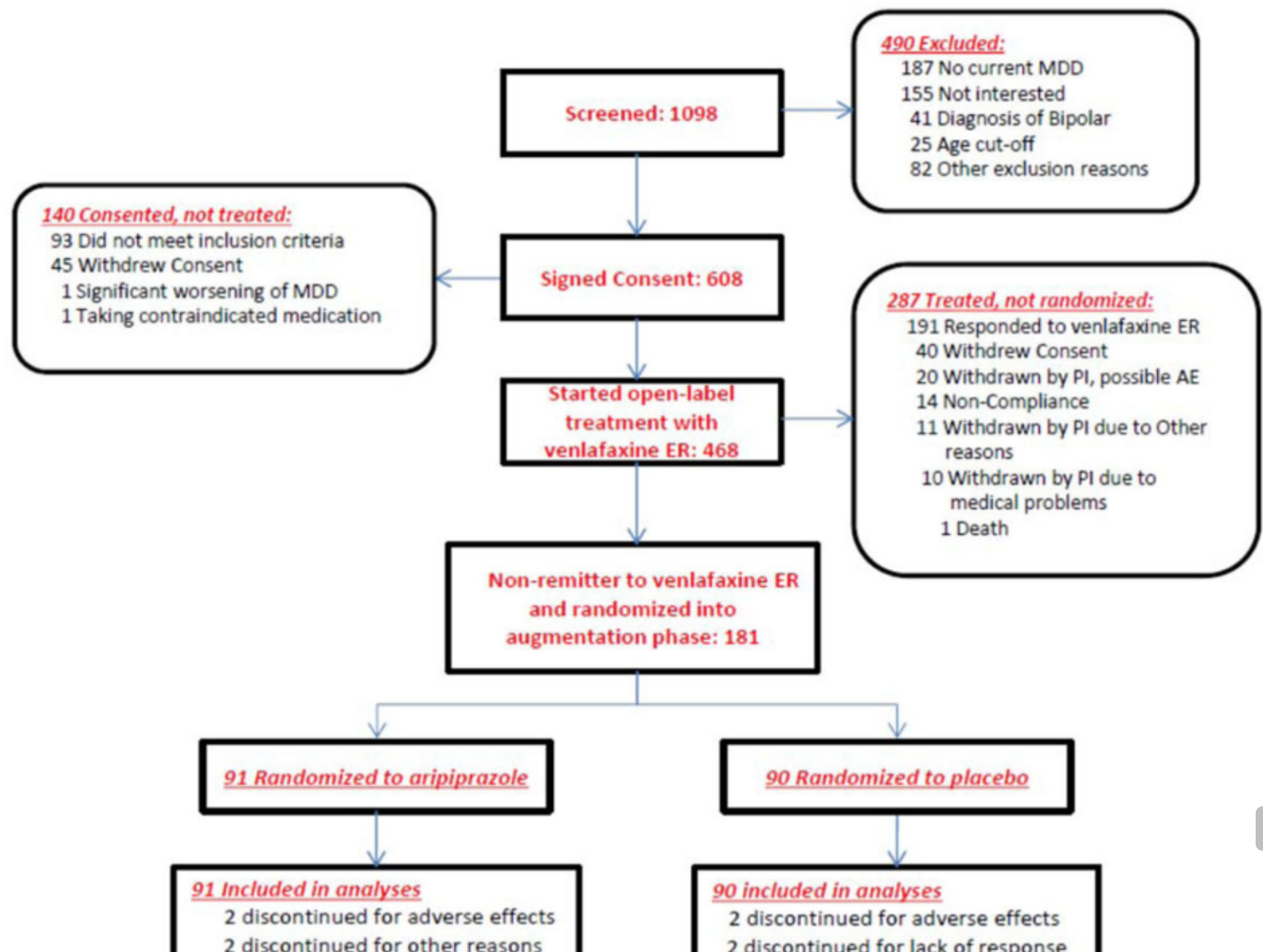
The correct answer is: 44% and 29%

Question 5

Not answered

Marked out of 1.00

In a clinical study, 468 participants older than 60 with current major depressive episode were initially treated with venlafaxine extended-release (ER); 96 (20.5%) did not complete this open phase, 191 (40.8%) remitted, and 181 (38.7%) did not remit and were randomized to 12 weeks of double-blind augmentation with aripiprazole or placebo. Remission, defined as Montgomery-Asberg Depression Rating Scale scores ≥ 10 , was assessed along with resolution of suicidal ideation, and safety and tolerability with cardiometabolic and neurological measures. Analyses were conducted according to the intention-to-treat principle. Older adults on aripiprazole had a higher remission rate than those on placebo (44% versus 29%; odds ratio [OR]=2.0, 95% CI 1.1–3.7, $p=0.03$; number needed to treat [NNT]=6.6 [95% CI 3.5–81.8]). Overall, remission was stable during 12 additional weeks of continuation treatment. (<http://europepmc.org/articles/pmc4690746>). The effectiveness of Aripiprazole is demonstrated by calculating the NNT for Aripiprazole compared to placebo augmentation. The NNT is



Select one:

- ☐ 9
- ☐ 5
- ☐ 7
- ☐ 11
- ☐ 3

40 completed as remitter
47 completed as nonremitter

3 discontinued for other reasons
26 completed as remitter
57 completed as nonremitter

40 Remitted and eligible for
continuation phase

26 Remitted and eligible for
continuation phase

Your answer is incorrect.

44% of aripiprazole-treated participants remitted while 28.9% of those on placebo remitted. The ABI is $44 - 28.9\% = 15.1\%$. So $NNT = 1/ABI = 1/0.151 = 6.6 \sim 7$.

The correct answer is: 7

38 Included in analyses
5 Relapsed
33 Did not relapse

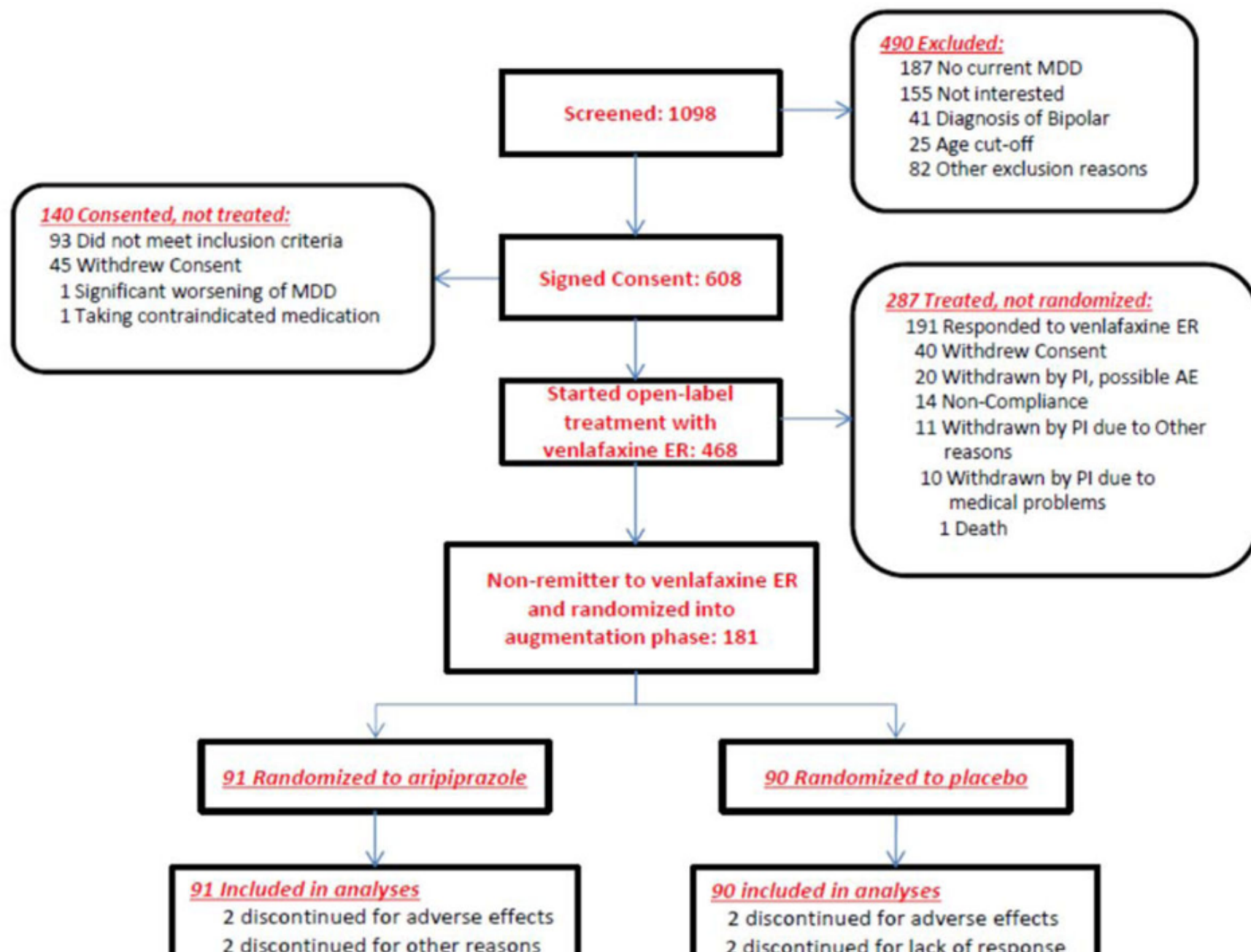
25 Included in analyses
2 Relapsed
23 Did not relapse

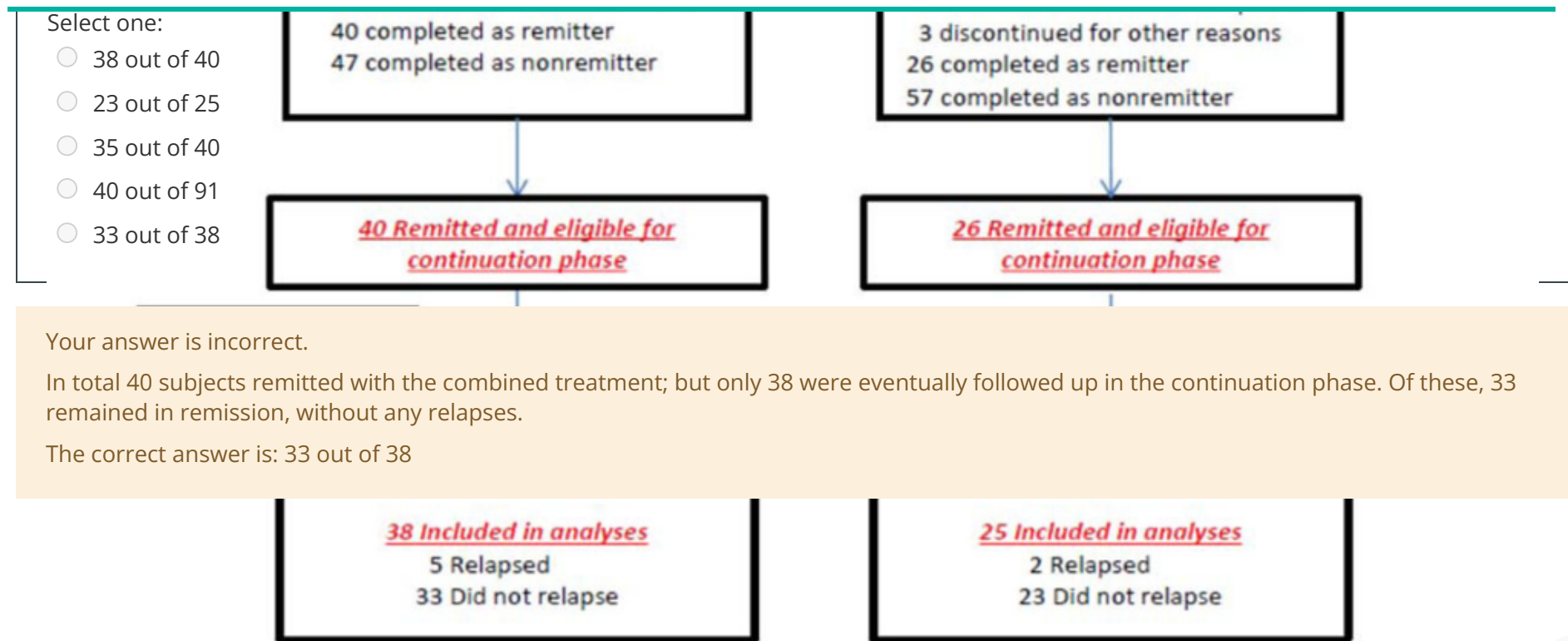
Question 6

Not answered

Marked out of 1.00

In a clinical study, 468 participants older than 60 with current major depressive episode were initially treated with venlafaxine extended-release (ER); 96 (20.5%) did not complete this open phase, 191 (40.8%) remitted, and 181 (38.7%) did not remit and were randomized to 12 weeks of double-blind augmentation with aripiprazole or placebo. Remission, defined as Montgomery-Asberg Depression Rating Scale scores ≥ 10 , was assessed along with resolution of suicidal ideation, and safety and tolerability with cardiometabolic and neurological measures. Analyses were conducted according to the intention-to-treat principle. Older adults on aripiprazole had a higher remission rate than those on placebo (44% versus 29%; odds ratio [OR]=2.0, 95% CI 1.1–3.7, $p=0.03$; number needed to treat [NNT]=6.6 [95% CI 3.5–81.8]). Overall, remission was stable during 12 additional weeks of continuation treatment. (<http://europepmc.org/articles/pmc4690746>). How many subjects in the treatment arm continued in remission during the continuation period?





Your answer is incorrect.

In total 40 subjects remitted with the combined treatment; but only 38 were eventually followed up in the continuation phase. Of these, 33 remained in remission, without any relapses.

The correct answer is: 33 out of 38

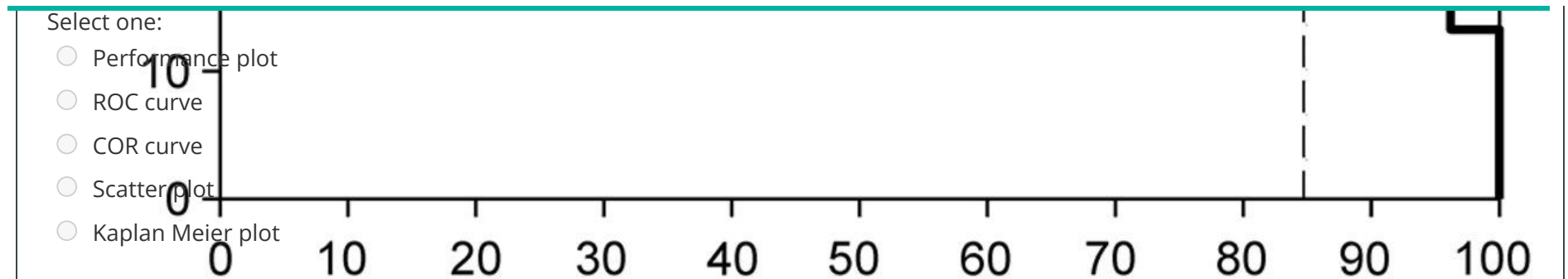
Question 7

Not answered

Marked out of 1.00

Visual processing abilities of patients with dementia with Lewy bodies (DLB) or Alzheimer disease (AD) dementia were assessed psychophysically using a simple horizontal motion discrimination task that engages the dorsal visual processing stream. Motion discrimination thresholds of patients with DLB and PDD were comparable and significantly higher (i.e., worse) than those of patients with AD dementia. The thresholds of patients with AD dementia and patients with PD were normal. These results were confirmed in subgroups of patients with DLB/PDD and AD dementia with autopsy-confirmed disease. The attached graph depicts a plot at various cut-off points, with a mark on the cut-off 0.23 that was recommended by the authors. Ref: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4626245/> . This graph is best described as





Your answer is incorrect.

This is a receiver operating characteristic curve but it is plotted with specificity on the x axis, rather than the conventional (1-specificity) or the false positivity rate. As a result, the curve is facing right.

The correct answer is: ROC curve



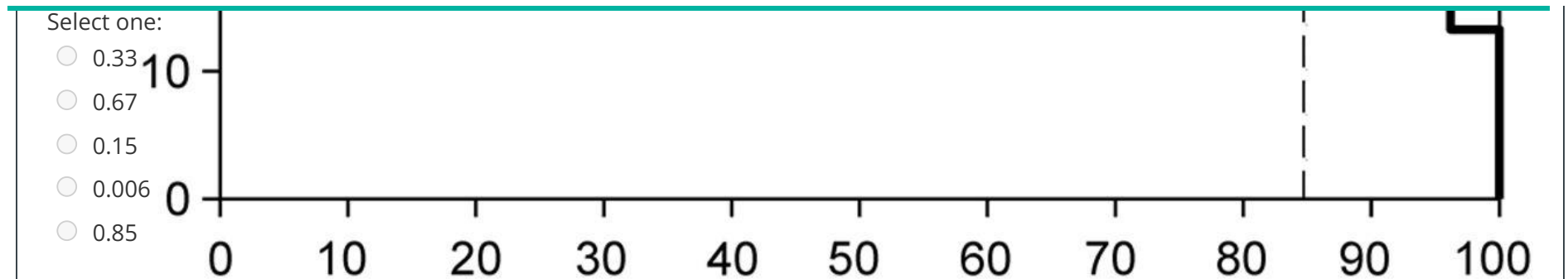
Question 8

Not answered

Marked out of 1.00

Visual processing abilities of patients with dementia with Lewy bodies (DLB) or Alzheimer disease (AD) dementia were assessed psychophysically using a simple horizontal motion discrimination task that engages the dorsal visual processing stream. Motion discrimination thresholds of patients with DLB and PDD were comparable and significantly higher (i.e., worse) than those of patients with AD dementia. The thresholds of patients with AD dementia and patients with PD were normal. These results were confirmed in subgroups of patients with DLB/PDD and AD dementia with autopsy-confirmed disease. The attached graph depicts a plot at various cut-off points, with a mark on the cut-off 0.23 that was recommended by the authors. Ref: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4626245/> . At the cut-off point highlighted in the graph, the true positivity rate is





Your answer is incorrect.

The motion discrimination threshold cut-off at 0.23 (star-mark) distinguished between DLB/PDD and AD dementia with 67% sensitivity (true-positive rate) and 85% specificity (true negative rate).

The correct answer is: 0.67



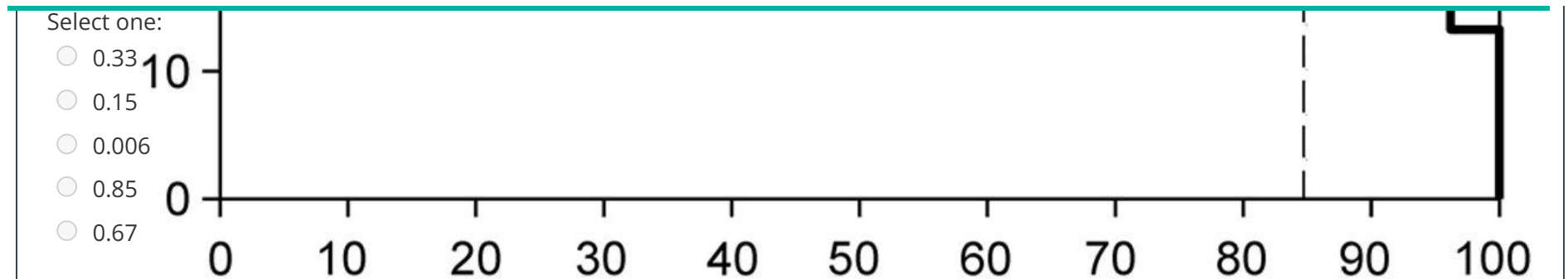
Question 9

Not answered

Marked out of 1.00

Visual processing abilities of patients with dementia with Lewy bodies (DLB) or Alzheimer disease (AD) dementia were assessed psychophysically using a simple horizontal motion discrimination task that engages the dorsal visual processing stream. Motion discrimination thresholds of patients with DLB and PDD were comparable and significantly higher (i.e., worse) than those of patients with AD dementia. The thresholds of patients with AD dementia and patients with PD were normal. These results were confirmed in subgroups of patients with DLB/PDD and AD dementia with autopsy-confirmed disease. The attached graph depicts a plot at various cut-off points, with a mark on the cut-off 0.23 that was recommended by the authors. Ref: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4626245/>. At the cut-off point highlighted in the graph, the true negativity rate is





Your answer is incorrect.

The motion discrimination threshold cut-off at 0.23 (star-mark) distinguished between DLB/PDD and AD dementia with 67% sensitivity (true-positive rate) and 85% specificity (true negative rate).

The correct answer is: 0.85



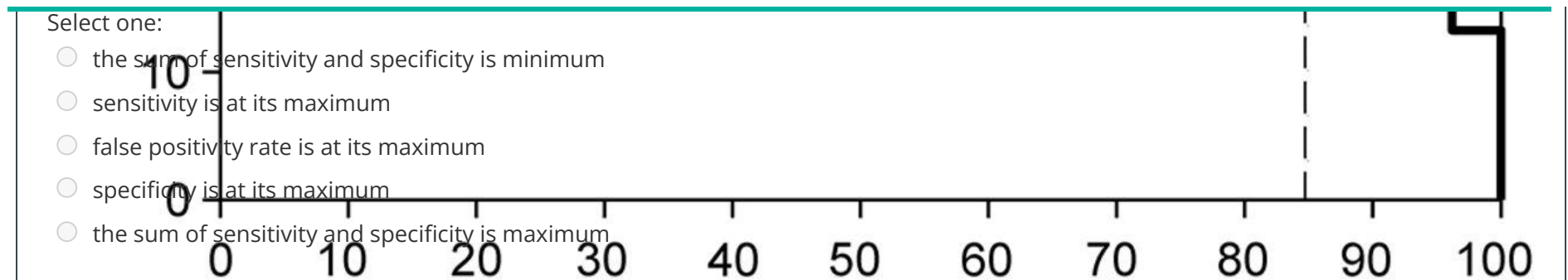
Question 10

Not answered

Marked out of 1.00

Visual processing abilities of patients with dementia with Lewy bodies (DLB) or Alzheimer disease (AD) dementia were assessed psychophysically using a simple horizontal motion discrimination task that engages the dorsal visual processing stream. Motion discrimination thresholds of patients with DLB and PDD were comparable and significantly higher (i.e., worse) than those of patients with AD dementia. The thresholds of patients with AD dementia and patients with PD were normal. These results were confirmed in subgroups of patients with DLB/PDD and AD dementia with autopsy-confirmed disease. The attached graph depicts a plot at various cut-off points, with a mark on the cut-off 0.23 that was recommended by the authors. Ref: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4626245/>. At the cut-off point highlighted in the graph:





Your answer is incorrect.

At the optimum cut-off point of a ROC curve, the sum of sensitivity and specificity is at its maximum; but this is not the point where either of them is at their highest.

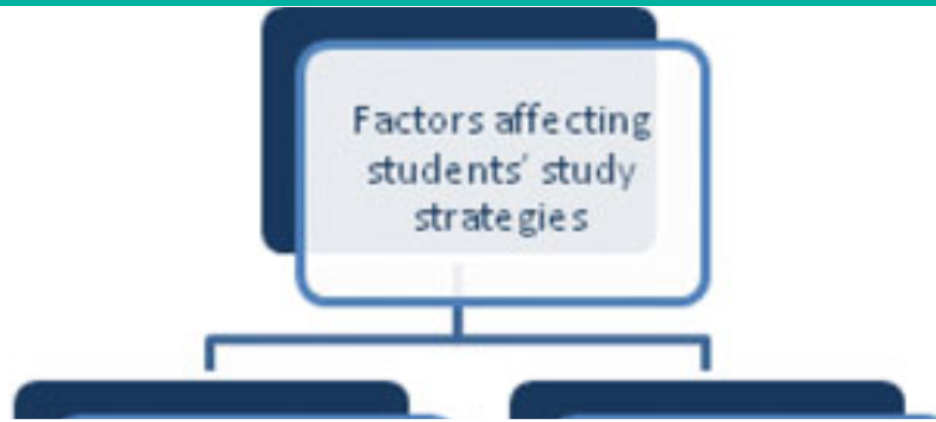
The correct answer is: the sum of sensitivity and specificity is maximum

Question 11

Not answered

Marked out of 1.00

To explore the effects of clinical supervision on the study strategies used by undergraduate medical students during their clinical rotations, a phenomenological study was carried out from November 2007 to December 2008. Semi-structured focus groups interviews for students in clinical years (3 and 4 of the 4 year programme) and individual interviews for teachers and students were conducted to explore students' and clinical teachers' perceptions and interpretations of factors influencing students' study strategies. All interviews were open-ended and were conducted by the principal investigator and a research assistant. Both students and teachers were allowed to talk freely and express their opinion on the effects of clinical supervision, patient encounters and assessment characteristics on students' adopted study strategies. For triangulation of the data, the authors conducted a second set of interviews with all of the 13 clinical teachers and 12 of the students who had participated in the focus groups. They were asked about their opinions on the results of the first round of students' focus group interviews. The obtained themes were used to analyse the whole dataset and rearrange the data according to the themes and sub-themes. All interviews were transcribed verbatim and themes and sub-themes relevant to the research questions were assigned and representing quotes were selected. This process was repeated for each interview in order to capture the full breadth and diversity of the students' and teachers' views and experiences. A Computer Software Atlas-ti (Version 5.2) was used to apply the themes to the whole dataset and to rearrange the data according to themes and sub- themes. Analysis of data from interviews with twenty-eight students and thirteen clinical supervisors yielded three major themes relating to factors affecting students' study strategies: "clinical supervisors and supervision", "stress and anxiety" and "assessment". The three themes we identified played a role in students' adoption of different study strategies in the "community of clinical practice". It appeared that teachers played a key role, particularly as assessors, clinical supervisors and as a source of stress to students. (<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3226698/>). What type of study is this?



Select one:

- ☐ Case control
- ☐ Uncontrolled trial
- ☐ Ecological
- ☐ Observational
- ☐ Qualitative

Your answer is incorrect.

Phenomenological studies are qualitative in nature. Other pointers in the precis include triangulation, focus groups, open-ended questions and semi-structured interviews.

The correct answer is: Qualitative

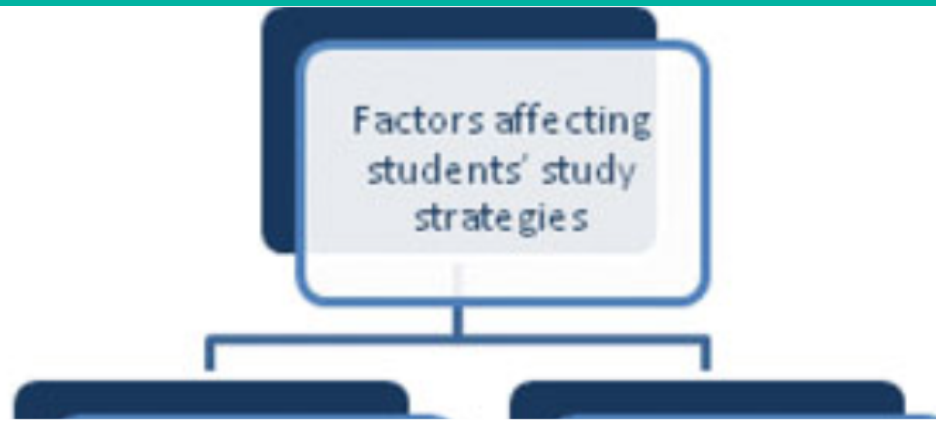


Question 12

Not answered

Marked out of 1.00

To explore the effects of clinical supervision on the study strategies used by undergraduate medical students during their clinical rotations, a phenomenological study was carried out from November 2007 to December 2008. Semi-structured focus groups interviews for students in clinical years (3 and 4 of the 4 year programme) and individual interviews for teachers and students were conducted to explore students' and clinical teachers' perceptions and interpretations of factors influencing students' study strategies. All interviews were open-ended and were conducted by the principal investigator and a research assistant. Both students and teachers were allowed to talk freely and express their opinion on the effects of clinical supervision, patient encounters and assessment characteristics on students' adopted study strategies. For triangulation of the data, the authors conducted a second set of interviews with all of the 13 clinical teachers and 12 of the students who had participated in the focus groups. They were asked about their opinions on the results of the first round of students' focus group interviews. The obtained themes were used to analyse the whole dataset and rearrange the data according to the themes and sub-themes. All interviews were transcribed verbatim and themes and sub-themes relevant to the research questions were assigned and representing quotes were selected. This process was repeated for each interview in order to capture the full breadth and diversity of the students' and teachers' views and experiences. A Computer Software Atlas-ti (Version 5.2) was used to apply the themes to the whole dataset and to rearrange the data according to themes and sub- themes. Analysis of data from interviews with twenty-eight students and thirteen clinical supervisors yielded three major themes relating to factors affecting students' study strategies: "clinical supervisors and supervision", "stress and anxiety" and "assessment". The three themes we identified played a role in students' adoption of different study strategies in the "community of clinical practice". It appeared that teachers played a key role, particularly as assessors, clinical supervisors and as a source of stress to students. (<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3226698/>). What is a major weakness in this study?



Select one:

- ☐ Using open-ended questions
- ☐ Using the same sample for both round 1 and round 2 interviews
- ☐ Using a small sample of subjects
- ☐ Including only those students in their clinical years (years 3 and 4)
- ☐ Restricting data collection to one calendar year

Your answer is incorrect.

Using the same sample for both round 1 and round 2 interviews has reduced the integrity of triangulation which the authors wanted to achieve in the first place. Instead, they could have used a different rater, or a different method of data collection, or a different sample to validate the results.

The correct answer is: Using the same sample for both round 1 and round 2 interviews

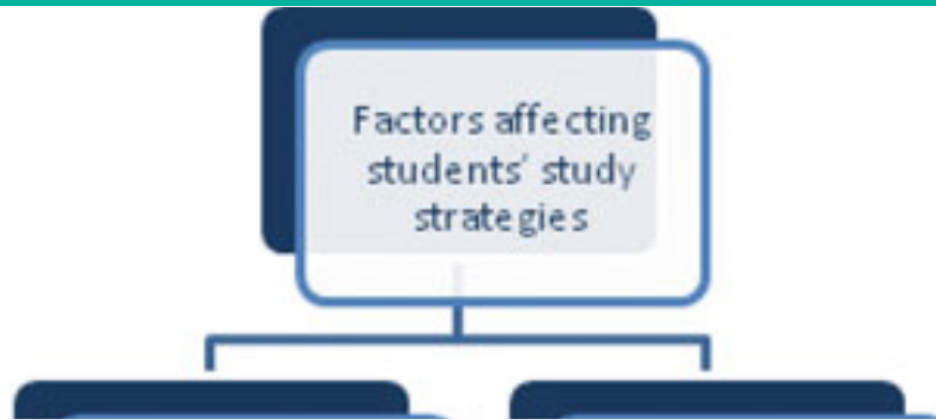


Question 13

Not answered

Marked out of 1.00

To explore the effects of clinical supervision on the study strategies used by undergraduate medical students during their clinical rotations, a phenomenological study was carried out from November 2007 to December 2008. Semi-structured focus groups interviews for students in clinical years (3 and 4 of the 4 year programme) and individual interviews for teachers and students were conducted to explore students' and clinical teachers' perceptions and interpretations of factors influencing students' study strategies. All interviews were open-ended and were conducted by the principal investigator and a research assistant. Both students and teachers were allowed to talk freely and express their opinion on the effects of clinical supervision, patient encounters and assessment characteristics on students' adopted study strategies. For triangulation of the data, the authors conducted a second set of interviews with all of the 13 clinical teachers and 12 of the students who had participated in the focus groups. They were asked about their opinions on the results of the first round of students' focus group interviews. The obtained themes were used to analyse the whole dataset and rearrange the data according to the themes and sub-themes. All interviews were transcribed verbatim and themes and sub-themes relevant to the research questions were assigned and representing quotes were selected. This process was repeated for each interview in order to capture the full breadth and diversity of the students' and teachers' views and experiences. A Computer Software Atlas-ti (Version 5.2) was used to apply the themes to the whole dataset and to rearrange the data according to themes and sub- themes. Analysis of data from interviews with twenty-eight students and thirteen clinical supervisors yielded three major themes relating to factors affecting students' study strategies: "clinical supervisors and supervision", "stress and anxiety" and "assessment". The three themes we identified played a role in students' adoption of different study strategies in the "community of clinical practice". It appeared that teachers played a key role, particularly as assessors, clinical supervisors and as a source of stress to students. (<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3226698/>). Identify one strength of this study



Select one:

- ☐ Using computerised random sampling
- ☐ Using the same interviewer for all interviews
- ☐ The size of the recruited sample
- ☐ Using quotations to identify major themes and subthemes
- ☐ Using a computer software for the analysis

Your answer is incorrect.

Using the same interviewer may help achieve consistency, but this comes at the cost of reducing varied perspectives in a qualitative research. Sample size is somewhat irrelevant to assess the quality of a qualitative study. The use of verbatim reports (quotes) is a specific strength that helps to preserve the original opinions without distortion during the process of analysis.

The correct answer is: Using quotations to identify major themes and subthemes

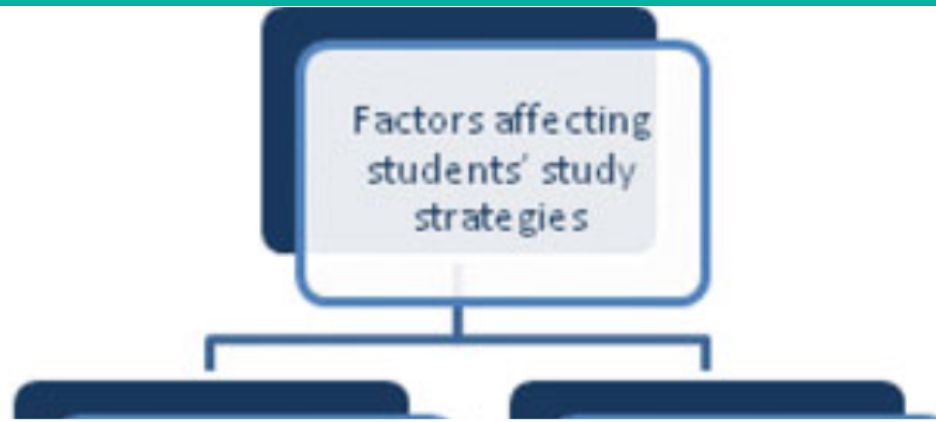


Question **14**

Not answered

Marked out of 1.00

To explore the effects of clinical supervision on the study strategies used by undergraduate medical students during their clinical rotations, a phenomenological study was carried out from November 2007 to December 2008. Semi-structured focus groups interviews for students in clinical years (3 and 4 of the 4 year programme) and individual interviews for teachers and students were conducted to explore students' and clinical teachers' perceptions and interpretations of factors influencing students' study strategies. All interviews were open-ended and were conducted by the principal investigator and a research assistant. Both students and teachers were allowed to talk freely and express their opinion on the effects of clinical supervision, patient encounters and assessment characteristics on students' adopted study strategies. For triangulation of the data, the authors conducted a second set of interviews with all of the 13 clinical teachers and 12 of the students who had participated in the focus groups. They were asked about their opinions on the results of the first round of students' focus group interviews. The obtained themes were used to analyse the whole dataset and rearrange the data according to the themes and sub-themes. All interviews were transcribed verbatim and themes and sub-themes relevant to the research questions were assigned and representing quotes were selected. This process was repeated for each interview in order to capture the full breadth and diversity of the students' and teachers' views and experiences. A Computer Software Atlas-ti (Version 5.2) was used to apply the themes to the whole dataset and to rearrange the data according to themes and sub- themes. Analysis of data from interviews with twenty-eight students and thirteen clinical supervisors yielded three major themes relating to factors affecting students' study strategies: "clinical supervisors and supervision", "stress and anxiety" and "assessment". The three themes we identified played a role in students' adoption of different study strategies in the "community of clinical practice". It appeared that teachers played a key role, particularly as assessors, clinical supervisors and as a source of stress to students. (<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3226698/>). The difference between the population average and the sample average is called



Select one:

- ☐ Variance
- ☐ Range
- ☐ Standard deviation
- ☐ Standard error of the mean
- ☐ Confidence Interval

Your answer is incorrect.

The extent to which a sample mean is likely to differ from the population mean is called the SEM or standard error of the mean.

The correct answer is: Standard error of the mean



Question 15

Not answered

Marked out of 1.00

For a random sample of 81 women, the average resting pulse rate is 72 beats per minute, and the sample standard deviation is $SD = 18$. The standard error of the sample mean is

Select one:

- ☐ 2.778
- ☐ 2
- ☐ 1.745
- ☐ 8
- ☐ 1

Your answer is incorrect.

The extent to which a sample mean is likely to differ from the population mean is called the SEM or standard error of the mean. SD and the SEM are related ($sem = sd / \sqrt{n}$). In this example, $SEM = 18 / \sqrt{81} = 18 / 9 = 2$

The correct answer is: 2



Question 16

Not answered

Marked out of 1.00

Which of the following symptom is most likely in a patient with severe and acute lithium toxicity?

Select one:

- ☐ Diplopia
- ☐ Coarse Tremors
- ☐ Pedal edema
- ☐ Nystagmus
- ☐ Achromtopsia

Your answer is incorrect.

Patients with acute lithium elevation predominantly suffer from nausea, vomiting, cramping, and sometimes diarrhea. Progression to severe toxicity can involve neuromuscular signs such as tremulousness (coarse tremors), dystonia, hyperreflexia, and ataxia. Toxic effects occur at levels of >1.5 mmol/L. Above 2 mmol/L, increased disorientation and seizures usually occur, which can progress to coma and ultimately death.

The correct answer is: Coarse Tremors

Question 17

Not answered

Marked out of 1.00

Which of the following is the most common ECG finding in a patient with acute lithium toxicity?

Select one:

- ☐ T wave flattening
- ☐ Deep Q waves
- ☐ QT prolongation
- ☐ Progressive increase in PR
- ☐ ST depression

Your answer is incorrect.

Lithium toxicity induces various ECG changes – the most common of these is nonspecific T-wave flattening followed by dysfunction of sinus node and prolonged QTc.

The correct answer is: T wave flattening

Question 18

Not answered

Marked out of 1.00

The most likely reason to transfer an anorexic patient in a psychiatric ward to a medical unit is

Select one:

- ☐ Jaundice
- ☐ Tachycardia
- ☐ BMI of 15
- ☐ Onset of fatigue
- ☐ QTc > 520

Your answer is incorrect.

Please be aware of the MARSIPAN report produced by the RCPsych Working Group (<http://www.rcpsych.ac.uk/files/pdfversion/CR189.pdf>) which provides comprehensive guidelines on managing very sick patients with anorexia nervosa. A significant increase in QT and QTc, can result from weight loss in patients with anorexia nervosa (Vázquez et al., 2003). Due to the elevated risk of sudden cardiac death, patients with QTc prolongation should be medically managed.

The correct answer is: QTc > 520

Question 19

Not answered

Marked out of 1.00

The most likely electrolyte abnormality of refeeding a grossly anorexic patient in a medical ward is

Select one:

- ☐ Hypercalcemia
- ☐ Hypermagnesaemia
- ☐ Hyponatremia
- ☐ Hyperkalemia
- ☐ Hypophosphatemia

Your answer is incorrect.

Excerpt from MARSIPAN report "In an acute medical service (Gaudiani et al, 2012) 45% of patients developed hypophosphataemia and in French intensive care units (Vignaud et al, 2010) 11% of patients developed re-feeding syndrome. It appears, therefore, that the highest rate of re-feeding syndrome is reported from the units in which patients are most unwell, namely adult medical and intensive care setting".

As noted in multiple sources, including MARSIPAN (Management of Really Sick Patients with Anorexia Nervosa), the potentially fatal refeeding syndrome is caused by the switch from fasting gluconeogenesis to carbohydrate-induced insulin release, inducing considerable intracellular uptake of potassium, phosphate, and magnesium from the bloodstream. This can cause profound biochemical derangements including hypokalaemia, hypophosphataemia, hypomagnesaemia, and occasionally hypocalcaemia.

Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021.

The correct answer is: Hypophosphatemia

Question 20

Not answered

Marked out of 1.00

What would you advise a patient on maximum dose mirtazapine and venlafaxine combination but still suffers from resistant depression?

Select one:

- ☐ Reduce venlafaxine and mirtazapine and add lithium
- ☐ Stop venlafaxine and start lithium
- ☐ Stop mirtazapine and start lithium
- ☐ Plan for ECT
- ☐ Add carbamazepine

Your answer is incorrect.

According to BAP guidelines, "consider adding quetiapine (A), aripiprazole (A) or lithium (A) as first-line treatments, and risperidone (A), olanzapine (B), tri-iodothyronine (B) or mirtazapine (B) as second-line treatments, being aware that the evidence derives mainly from studies in which lithium and tri-iodothyronine were added to TCAs and the other drugs added to SSRI/SNRIs". Assuming that lithium augmentation of a SSRI/SNRI has not been tried yet, we can choose to add lithium to venlafaxine.

The correct answer is: Stop mirtazapine and start lithium

Question **21**

Not answered

Marked out of 1.00

What is the chance of 40-year-old mother giving birth to a child with Down syndrome?

Select one:

- ☐ 1 in 200
- ☐ 1 in 25
- ☐ 1 in 100
- ☐ 1 in 50
- ☐ 1 in 350

Your answer is incorrect.

With increasing age, the risk of giving birth to a child with Down syndrome greatly increases (2 to 3 times increase in risk every 5 years). By age 35, the absolute risk for a pregnant mother 1 in 350. By age 40, it bounces to 1 in 100, and by age 45, it peaks to 1 in 30. Ref: <https://ghr.nlm.nih.gov/condition/down-syndrome>

The correct answer is: 1 in 100

Question **22**

Not answered

Marked out of 1.00

A girl who attends a mainstream school with apparently normal IQ has visuospatial difficulties. The most likely diagnosis is

Select one:

- ☐ Rett syndrome
- ☐ Foetal alcohol syndrome
- ☐ Angelman's syndrome
- ☐ Fragile-X syndrome
- ☐ Turners syndrome

Your answer is incorrect.

Turner syndrome is relatively common (1/2,500 women) in a phenotypic female. It is associated with a cognitive profile of normal intellectual and verbal function but impairments in visual-spatial, executive, and social cognitive domains.

The correct answer is: Turners syndrome



Question **23**

Not answered

Marked out of 1.00

A patient who had a severe episode of herpes encephalitis 3 months ago recovered fully but now suffers from new-onset carbohydrate craving. The most likely diagnosis is:

Select one:

- ☐ Cushing syndrome
- ☐ Hyperthyroidism
- ☐ Diabetes mellitus
- ☐ Kluver-Bucy syndrome
- ☐ Prader Willi Syndrome

Your answer is incorrect.

Hyperorality, hypersexuality, and abnormal eating behaviour characterized by carbohydrate craving after herpes encephalitis is suggestive of Kluver-Bucy syndrome resulting from medial temporal lobe damage.

The correct answer is: Kluver-Bucy syndrome

Question **24**

Not answered

Marked out of 1.00

What is most likely drug associated with gum bleeding?

Select one:

- ☐ Carbamazepine
- ☐ fluoxetine
- ☐ Sodium Valproate
- ☐ Lithium
- ☐ Olanzapine

Your answer is incorrect.

Non-GI bleeds such as gum bleeds and subconjunctival bleeds are rare with SSRIs. Carbamazepine can cause gingival bleeds due to their propensity to reduce platelet levels. Other antiepileptic drugs that can cause gingival bleed include levacetam and phenytoin.

The correct answer is: Carbamazepine



Question **25**

Not answered

Marked out of 1.00

Which of the following medications can cause fatal intestinal obstruction?

Select one:

- ☐ Selegiline
- ☐ Atomoxetine
- ☐ Buspirone
- ☐ Varenicline
- ☐ Clozapine

Your answer is incorrect.

Rapidly fatal intestinal obstruction has been reported with clozapine, sometimes even without prior warning signs. Some case reports also implicate risperidone and olanzapine in intestinal obstructions.

The correct answer is: Clozapine



Question **26**

Not answered

Marked out of 1.00

Ebstein's anomaly is sometimes associated with drugs other than lithium. Choose one such drug:

Select one:

- ☐ Methadone
- ☐ Diazepam
- ☐ Duloxetine
- ☐ Carbamazepine
- ☐ Paroxetine

Your answer is incorrect.

Genetic, reproductive, and environmental risk factors can contribute to Ebstein's anomaly. It is more common in twins, in those with a family history of congenital heart disease, and in those with maternal exposure to lithium or benzodiazepines. But most cases are sporadic. Ref: <http://circ.ahajournals.org/content/115/2/277> <https://medlineplus.gov/ency/article/007321.htm>

The correct answer is: Diazepam

Question **27**

Not answered

Marked out of 1.00

A major aim of interpersonal therapy for depression is

Select one:

- ☐ Aid in conflict resolution
- ☐ Reduce stigma
- ☐ Assist in problem solving
- ☐ Managing transference
- ☐ Promote a gain in insight

Your answer is incorrect.

Resolving an interpersonal struggle in a role dispute is one of the primary aims of IPT. Whether life events follow or precede mood changes, the patient's task in therapy is to resolve the disturbing life event(s), building social skills and helping to organize his or her life. Excerpt from <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC1414693/>

The correct answer is: Aid in conflict resolution

Question **28**

Not answered

Marked out of 1.00

The three R's of Cognitive Analytical Therapy are:

Select one:

- ☐ Reformulation, Recovery, Revision
- ☐ Recognition, Recovery, Revision
- ☐ Recognition, Role Reversal, Recovery
- ☐ Reformulation, Recognition, Revision
- ☐ Reformulation, Role Reversal, Revision

Your answer is incorrect.

The three R's of Cognitive Analytical Therapy are Reformulation, Recognition and Revision.

The correct answer is: Reformulation, Recognition, Revision



Question 29

Not answered

Marked out of 1.00

The type of family therapy that depends on transactions is:

Select one:

- ☐ Functional Family Therapy
- ☐ Behavioural Couples Therapy
- ☐ Strategic Family Therapy
- ☐ Systemic Family Therapy
- ☐ Structural Family Therapy

Your answer is incorrect.

A family establishes a set of rules for its daily functioning through its transactions. According to structural family therapy, these rules form a family unit's structure.

The correct answer is: Structural Family Therapy



Question 30

Not answered

Marked out of 6.00

Psychometrics of rating scales

Which of the listed terms best describes the attributes of the scales that are being tested in each of the following scenarios?

1. You are administering a putative test of cognition and MMSE (mini-mental state examination) at the same time, and the correlation between the two is computed ✖
2. A new self-report questionnaire for PTSD called NEO-PTSD has questions covering the three major dimensions of PTSD - hyperarousal symptoms, avoidance and intrusions - in a detailed manner. ✖
3. A medical student has noted that a specific scale has been repeatedly used in several controlled trials evaluating treatments for early onset schizophrenia. He is now keen to know the extent to which the observations using this scale can be generalized to a wider population of patients. ✖
4. The extent to which the same trait is measured by different methods ✖
5. The extent to which an instrument distinguishes the various distinct constructs that may relate to each other ✖
6. A research team wants to assess whether the acute stress disorder related dissociation in trauma-exposed youth accounts for later PTSD in adult life. ✖

Explanation: 1) Concurrent validity reflects the agreement of a new instrument with another scale, which has itself been shown to be valid. It is usually determined by comparing the new scale within an external gold standard.
2) Content validity refers to whether the contents i.e. each sub-scales, items or elements of the test are in line with the general objectives or specifications the test was originally designed to measure. A good coverage of all domains thought to be related to the measured condition would improve the content validity of an instrument.
3) External validity describes the extent to which the results from a study can be generalized to a wider population.
4) When two instruments produce the same result, they are probably measuring the same construct or trait; thus they are likely to be

convergent.
5) If an instrument has good divergent validity, it will be able to accurately discriminate between overlapping constructs. For example, Hamilton Anxiety Scale will be said to have high divergent validity, if it rates symptoms seen in a patient with Generalised Anxiety Disorder consistently higher than the symptoms seen in a patient with depression and some features of anxiety. 6) When testing predictive validity, we assess an instrument's ability to predict something it should theoretically be able to predict. In this example, the authors expect dissociation to predict later PTSD.



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